

Jot — Clinical Longitudinal Report

Patient Report Generated: Jun 23, 2026

USER SUMMARY

This report analyzes the longitudinal data for a **22-year-old male managing POTS, MCAS, Ehlers-Danlos Syndrome, and Hypermobility Syndrome**. His baseline normal state, based on recent data, includes an average monthly heart rate of 62 bpm, a daily average of 22.1 orthostatic heart rate spikes, and frequent experiences of migraines, fatigue, and rib/back pain. He is currently 13 days away from his next specialist appointment with Brad.

TEMPORAL TRENDS & MAJOR OUTLIERS

Over the past 30 days, the user's average daily heart rate has remained around 62-66 bpm, consistent with the overall monthly average. However, there are notable fluctuations in other metrics.

- **Sleep Duration:** The average sleep duration over the last 30 days is approximately 7 hours and 44 minutes. However, several days show significant deviations:
- **Major Outlier (Low Sleep):** June 17, 2026, with only 3 hours and 33 minutes of sleep. This was followed by reports of heart palpitations and left rib pain with difficulty breathing.
- **Major Outlier (Low Sleep):** May 27, 2026, with only 3 hours and 10 minutes of sleep, preceding reports of heart palpitations, light-headedness upon standing, low stamina, and headache.
- **Major Outlier (Low Sleep):** May 26, 2026, with 4 hours and 8 minutes of sleep, coinciding with the start of Ivabradine, back pain, and headaches upon standing.
- **Heart Rate Variability (HRV):** HRV shows considerable daily variation, ranging from 44 ms on June 21, 2026, to 89 ms on June 8, 2026. Lower HRV can indicate increased stress or autonomic imbalance. The lowest recorded HRV of 44 ms on June 21, 2026, was followed by a log of taking a 2-hour nap and waking up even more tired, along with right rib pain.
- **Orthostatic Heart Rate Spikes:** The user consistently experiences orthostatic spikes, with an average of 22.1 events per day where the heart rate jumps 30 bpm or more above baseline. The average maximum jump was +94 bpm.

- **Major Outlier (Extreme Spike):** June 20, 2026, recorded a maximal heart rate of 173 bpm from a 59 bpm baseline (a jump of +114 bpm), with 41 events. This day also reported a headache and heart palpitations.
- **Major Outlier (Extreme Spike):** June 11, 2026, recorded a maximal heart rate of 168 bpm from a 53 bpm baseline (a jump of +115 bpm), with 25 events. This correlated with feeling very tired and lacking energy, along with heart palpitations and left rib pain.

TRIGGER LOGIC & CORRELATIONS

- **High Heart Rate and Migraines/Headaches:** The user consistently reports migraines or headaches coinciding with or following days of high heart rate spikes. The user's personal context states migraines show up specifically every time the heart rate goes over 160 bpm.
 - On June 20, 2026, a max HR of 173 bpm was recorded, and the user logged "Headache starting" and "Heart Palpitations."
 - On June 19, 2026, a max HR of 167 bpm was recorded, followed by a log of "Migraine."
 - On June 11, 2026, a max HR of 168 bpm was recorded, and the user logged "Heart Palpitations" and "feeling very tired and like I don't have energy today."
- **Postural Changes and Symptoms (POTS flares):** Standing up, rolling over, or making the bed frequently triggers heart palpitations, light-headedness, and headaches, directly aligning with POTS symptoms.
 - "Heart palpitations every time I roll over" (May 29, 2026) and "strong heart palpitations when rolling over" (May 31, 2026).
 - "stood up and got light-headed" (May 27, 2026) and "headache spike when standing" (May 26, 2026).
 - "making my bed put me at a 150bpm" (March 26, 2026).
 - "Heart palpitations — When standing" (May 10, 2026).
- **Physical Exertion and Symptoms:** Activities leading to increased heart rate or physical strain are consistently associated with fatigue, shortness of breath, and pain.
 - "high heart rate when I walked up stadium stairs. Out of breath" (May 22, 2026).
 - "out of breath after going downstairs and back up" (April 25, 2026).
 - "Almost passed out in the shower. Super drained now" (April 19, 2026).
 - "Getting out of the hot tub almost made me pass out" (March 26, 2026).

- **Rib and Back Pain with Physical Therapy/Scoliosis:** The logs frequently mention rib and back pain, often exacerbated by physical therapy or specific postures, directly linked to the Ehlers-Danlos Syndrome and scoliosis diagnosis.
- "Scoliosis pain" (June 23, 2026), "Right Rib Pain" (June 21, 2026), "Hurts to Breathe · Left Rib Pain" (June 18, 2026).
- "My back hurts after physical therapy. We did a lot of exercises and he said it will most likely hurt for the next couple days" (March 27, 2026).
- "Physical therapist said he wants me to do 3 exercises every day for the next 3 weeks to see if we can help my ribs" (April 17, 2026).
- **Medication Changes and Impact:** Recent medication adjustments have had a noticeable impact.
- **Carvedilol cessation:** The doctor instructed to stop Carvedilol on March 25, 2026, to see if nausea resolved, and then confirmed to stop it entirely on April 2, 2026. The logs around this time show ongoing nausea, headaches, and general fatigue, making it difficult to definitively link the resolution of nausea to Carvedilol cessation alone.
- **Qulipta initiation and cessation:** Started Qulipta for migraines on April 7, 2026. Stopped on May 4, 2026, as it "wasn't truly helpful." Migraines continued to be reported frequently both before and after this period.
- **Ivabradine initiation:** Started taking Ivabradine on May 26, 2026. By June 4, 2026, the user noted "it seems like the ivabradine is helping a little but it makes me tired." This suggests a potential trade-off between heart rate control and fatigue.
- **Creatine initiation:** Started taking creatine on June 4, 2026. No immediate significant changes in heart rate or symptom frequency are evident in the logs directly after this, though sustained monitoring is needed.

STRATEGIC INQUIRY FOR Brad

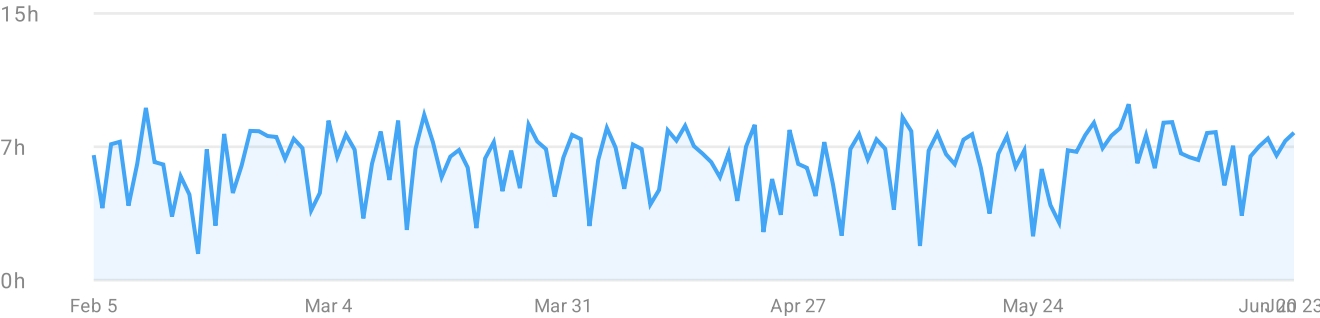
- Given the consistent occurrence of heart rate spikes exceeding 160 bpm and the user's correlation of these spikes with migraines, how effective is the current Ivabradine dosage in preventing these high spikes, and should a dosage adjustment or alternative strategy be considered to further mitigate them?
- The user continues to experience frequent rib and back pain, despite physical therapy exercises. Could the recent bubble study (PFO) results (from April 16, 2026) provide any insights into potential cardiovascular contributions to the chest discomfort, and what further diagnostic steps might be warranted for the persistent rib pain, especially "hurts to breathe" entries?

- The user noted that Ivabradine is "helping a little but it makes me tired." What is the long-term strategy for managing this fatigue, and are there ways to optimize the heart rate control without exacerbating tiredness, perhaps by adjusting the timing or other supportive measures?
- Since starting creatine on June 4, 2026, the user has only logged "Started creatine" on June 5, 2026. To assess its impact, we recommend a **4-week monitoring period** to track any changes in energy levels, symptom frequency (especially fatigue and muscle pain), and orthostatic heart rate trends. Are there specific biomarkers or symptoms you'd like the user to focus on tracking during this period?

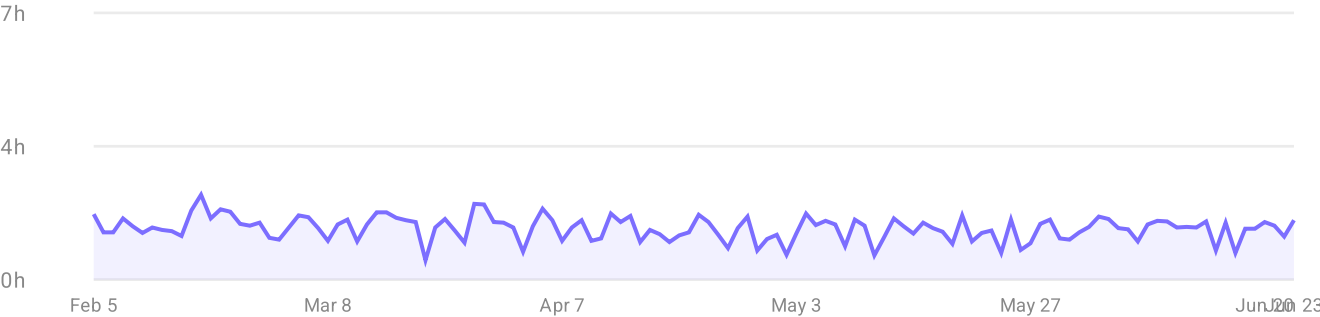
Disclaimer: This report is generated by AI based on personal logs and should be reviewed by a medical professional.

Longitudinal Biometrics & Tracker Charts

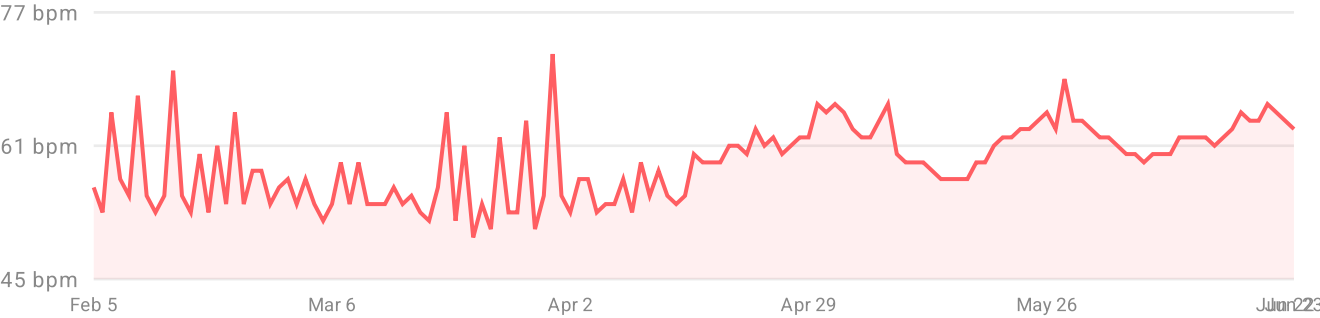
Sleep Duration



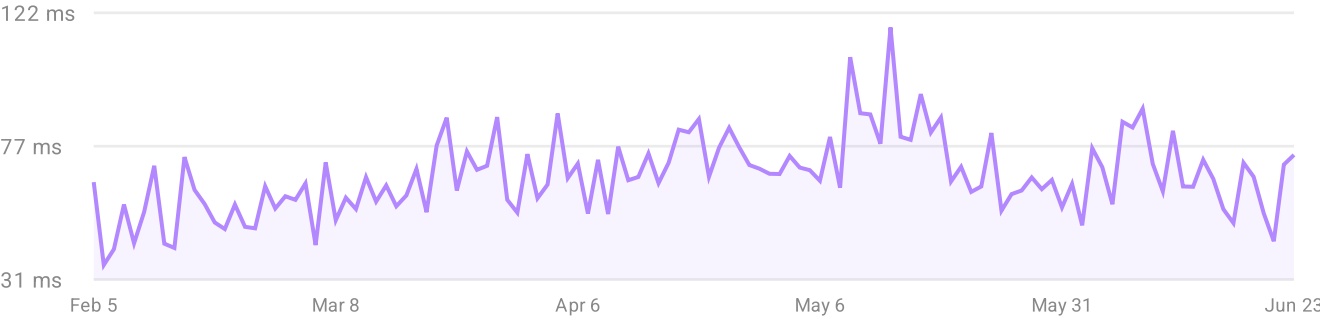
Deep Sleep



Avg Heart Rate

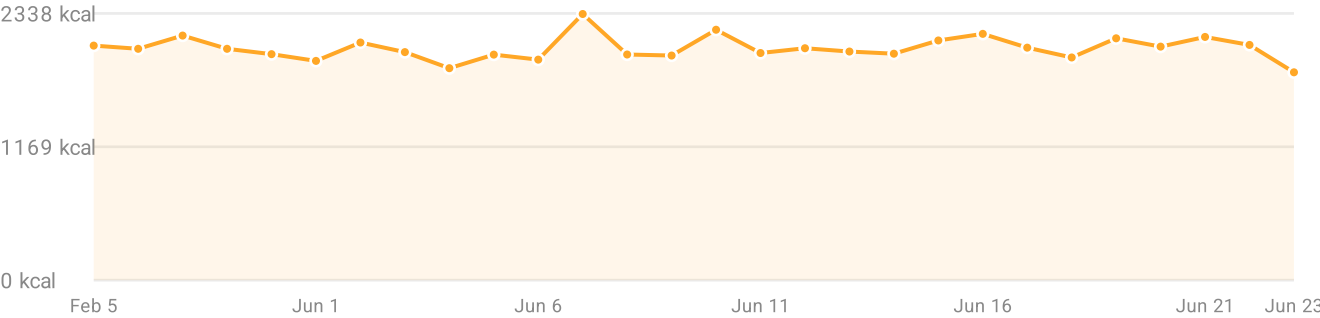


HRV (RMSSD)

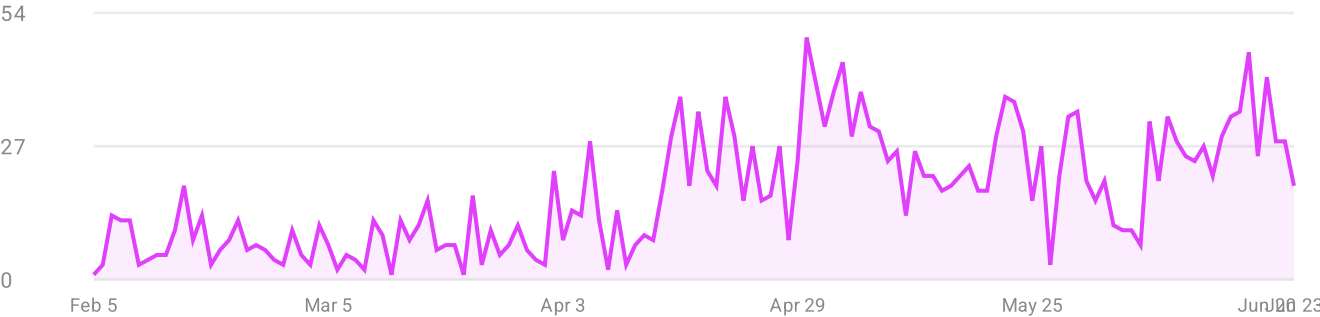


Longitudinal Biometrics & Tracker Charts (Cont.)

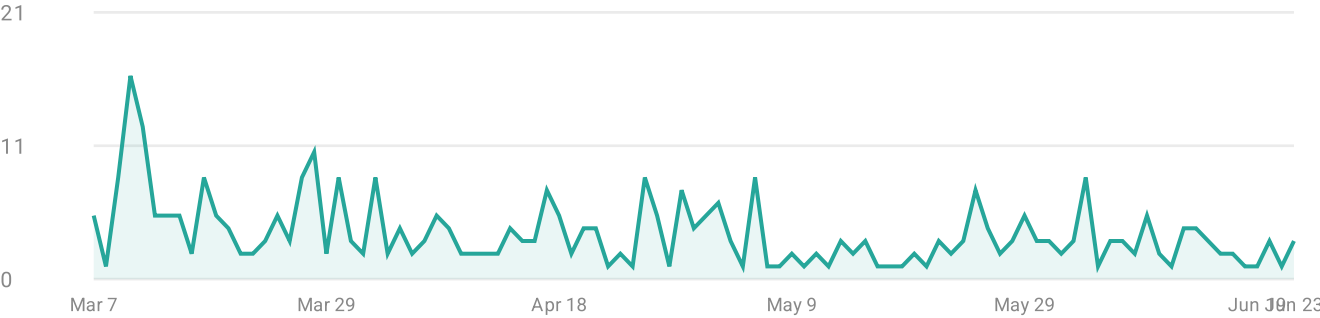
Calories Burned



HR Spikes



Number of Jots



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